

Clinical Protocol

Protocol Number: IMMUNO-MEL-201

Study: A Phase II Randomized, Multicenter Study Evaluating ImmunoX Plus Standard Therapy Versus Standard Therapy Alone in Advanced Melanoma.

Portfolio Notice: This is a fictional protocol created for educational and interview portfolio purposes.

1. Background

Advanced melanoma remains a life-threatening disease despite recent advances in immunotherapy. This fictional study evaluates the efficacy and safety of a novel immunotherapy (ImmunoX) when added to standard therapy.

2. Objectives

Primary: Compare progression-free survival (PFS). Secondary: Evaluate overall response rate (ORR), overall survival (OS), duration of response, safety, tolerability and quality of life.

3. Study Design

Phase II, randomized (1:1), open-label, multicenter, approximately 120 participants enrolled across five international sites. Visits: Screening, Baseline, Weeks 4, 8, 12, 18, 24, End of Treatment, and 30-day Safety Follow-up.

4. Study Population

Adults aged 18 years or older with histologically confirmed unresectable or metastatic melanoma, ECOG Performance Status 0–1, measurable disease according to RECIST v1.1.

5. Inclusion Criteria

1. Age ≥ 18 years. 2. Signed informed consent. 3. Histologically confirmed advanced melanoma. 4. ECOG 0–1. 5. Adequate organ function. 6. Measurable disease.

6. Exclusion Criteria

Prior ImmunoX exposure, uncontrolled infection, active autoimmune disease requiring systemic treatment, pregnancy or breastfeeding, participation in another interventional study.

7. Study Procedures

Screening includes informed consent, demographics, medical history, eligibility review, laboratory assessments and baseline tumour assessment. Follow-up visits include vitals, labs, treatment administration, concomitant medications, adverse event review and tumour assessments at protocol-defined intervals.

8. Safety Reporting

All adverse events (AEs) will be collected from first dose until 30 days after treatment discontinuation. Serious adverse events (SAEs) will be reported immediately according to sponsor procedures. Severity will use CTCAE v5.0.

9. Data Management

Electronic data capture will be performed in REDCap using longitudinal events, edit checks, branching logic, repeating instruments, audit trails and role-based access. Data cleaning will include programmed edit checks and manual review. Queries will be tracked until resolution before database lock.

10. Statistical Considerations

Primary efficacy analyses will use the intent-to-treat population. Safety analyses will include all treated participants. Descriptive statistics will summarize baseline characteristics and safety outcomes.

11. Ethics

The study will comply with ICH E6 Good Clinical Practice, the Declaration of Helsinki and applicable local regulations. Ethics approval and written informed consent are required before study-specific procedures.

12. End of Study

The study concludes after database lock, statistical analysis and preparation of the clinical study report.